FINANCE AND CLAIMS

Bill what you did. Collect what you are owed.

Exact money, rules held as data, and views that show what has not been billed and what has not been paid. Nothing bills a patient by itself.

Billing

Money	Exact amounts with two decimals; tax rules and price lists as data; payments cannot exceed the balance; payments are idempotent; refunds are explicit.
Missing charges	A list of dispenses, completed stays, signed visits, completed lab orders and reported imaging that are on no live invoice. One action adds them to a draft invoice for the same patient; the same event cannot be billed twice.
Charge rules	Which catalogue item a visit, a test or an imaging modality is billed as, set by your billing team.
Payment terms	A due date when an invoice is issued, from terms your administrator sets. No default ships. Overdue invoices are listed.

Collections

Ageing	Unpaid invoices by age since issue (0 to 30, 31 to 60, 61 to 90, over 90 days), net collected per day and per method, the ten largest balances.
Claims by payer	Open claims by payer, claimed and approved-not-settled, with the oldest age.

Insurance

Pre-authorisation	Requests and decisions are recorded and decided by your staff. There is no electronic exchange with payers.
Claim cases	A checklist of what the payer needs, a frozen packet version, a receipt of how it was sent, payer queries tracked, and a claims assistant that proposes next steps and drafts a reply for staff to edit. It cannot submit.

What it does not do. No payment gateway is connected (payment links are a labelled simulator). Submission to a payer is recorded, not performed. Tax rules, prices and terms are yours to enter and to confirm with your accountant; the software ships no tax advice and no default terms.

FINANCE AND CLAIMS, IN THE NEXT RELEASE

Rules written down, exact to the paisa.

Built and tested, not yet in a release. All amounts are whole minor units; every rule is data the hospital controls; nothing contacts a payer, a tax authority or a scheme.

MODULE	WHAT IT DOES	ITS BOUNDARIES
Tariffs and estimates	Versioned payer rate cards, a self-pay price list by room class and a labelled non-binding estimate.	An estimate stores nothing beyond its audit entry.
Insurance eligibility	Eligibility check records (typed in from a payer portal or call, or simulated from the policy dates held here), payer rules per payer and service category, and an order-time check of the patient's and payer's share with warnings.	Stores nothing and sends nothing: no 270/271, NPHIES or health-card transport, and no promise of payment.
Fee waivers and exemptions	Statutory entitlements with evidence, two-person approval of discounts and waivers, a deduction order, a pre-discharge check and an exemption report.	Requester cannot approve their own request. Paid or void invoices are refused.
Mobile money	Receipts by provider code, statement matching and a daily till report.	No gateway: nothing contacts a provider. Receipts are never edited.
Tax invoices	Tax IDs and a tax category on every line of an issued invoice, chained credit notes in their own gap-free series, and unsigned JSON or UBL-like exports through profiles kept as data.	Never stamps, signs or clears anything with a tax authority. Split insurer and patient invoices are not built.
Payer claim files	Payer file layouts kept as versioned data, batches checked against the format, offline remittance import, and a denial queue with an appeal trail.	Contacts no payer. No named payer specifications or EDI output yet.
DRG grouper workbench	An operator-supplied, versioned rule table groups discharged admissions; case-mix index and length of stay against benchmark.	Labelled "Model grouper, not an official grouper".
Government scheme billing	Schemes with co-pay, caps and packages; enrolment with an authorisation; splitting an issued invoice into scheme and patient portions; balance alerts.	Nothing is submitted to a scheme. Excess over a cap falls back on the patient.